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UNIFIED HEALTHCARE SYSTEM

CLINICAL SUMMARY REPORT

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PATIENT INFORMATION

Patient Name: John Michael Anderson

Date of Birth: March 15, 1975

Gender: Male

MRN: PAT-2026-087543

Phone: (555) 867-5309

Email: john.anderson@email.com

Address: 456 Oak Street, Apt 12B, Chicago, IL 60614

Insurance: Aetna PPO

Member ID: AET-9876543210

Primary Care: Dr. Sarah Martinez, MD

DATE OF SERVICE: April 22, 2026

VISIT TYPE: Annual Wellness Examination

PROVIDER: Dr. Sarah Martinez, MD

CLINIC: Downtown Family Medicine

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VITAL SIGNS

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Blood Pressure: 142/88 mmHg (elevated)

Heart Rate: 78 bpm (regular rhythm)

Respiratory Rate: 16 breaths/min

Temperature: 98.4 °F (oral)

Oxygen Saturation: 97% on room air

Weight: 198 lb (89.8 kg)

Height: 5'10" (178 cm)

BMI: 28.4 kg/m² (overweight)

Pain Score: 2/10 (mild lower back discomfort)

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CURRENT DIAGNOSES

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1. Type 2 Diabetes Mellitus without complications (ICD-10: E11.9)

- Diagnosed: 2019
- Currently managed with oral medications
- Last A1c: 7.2% (February 2026)

2. Essential Hypertension (ICD-10: I10)

- Diagnosed: 2017

- Blood pressure not at goal today
- Will adjust medication

3. Hyperlipidemia, unspecified (ICD-10: E78.5)

- Diagnosed: 2020
- On statin therapy
- LDL trending down

4. Obesity (ICD-10: E66.9)

- BMI 28.4
- Discussed weight management strategies

5. Chronic Low Back Pain (ICD-10: M54.5)

- Intermittent, mild severity
- Managed conservatively

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CURRENT MEDICATIONS

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1. Metformin 1000mg - Take 1 tablet by mouth twice daily with meals

- For diabetes management
- Tolerating well, no GI side effects

2. Lisinopril 20mg - Take 1 tablet by mouth once daily in the morning

- For blood pressure control
- INCREASING TO 40mg due to elevated BP today

3. Atorvastatin 40mg - Take 1 tablet by mouth once daily at bedtime

- For cholesterol management
- No muscle aches reported

4. Aspirin 81mg - Take 1 tablet by mouth once daily

- For cardiovascular protection
- Taking with food

5. Metoprolol Succinate 50mg - Take 1 tablet by mouth once daily

- For heart rate control and BP
- Adding today for better BP control

6. Januvia (Sitagliptin) 100mg - Take 1 tablet by mouth once daily

- For additional diabetes control
- Started February 2026

7. Vitamin D3 2000 IU - Take 1 capsule by mouth once daily

- For vitamin D deficiency
- Levels normalizing

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ALLERGIES

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1. PENICILLIN - SEVERE

Reaction: Anaphylaxis (throat swelling, difficulty breathing)

First Occurrence: 1998

Avoid all penicillin-class antibiotics

2. SULFA DRUGS (Sulfamethoxazole/Trimethoprim) - MODERATE

Reaction: Skin rash, hives

First Occurrence: 2010

3. SHELLFISH (Shrimp, Crab, Lobster) - MODERATE

Reaction: Hives, facial swelling

Food allergy - carries EpiPen

4. LATEX - MILD

Reaction: Contact dermatitis

Use non-latex gloves for procedures

5. IBUPROFEN - MILD

Reaction: Stomach upset, GI bleeding risk

Use acetaminophen instead for pain

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LABORATORY RESULTS

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Date Collected: April 22, 2026

METABOLIC PANEL:

- Glucose (fasting): 126 mg/dL (H - Reference: 70-100)
- Hemoglobin A1c: 7.2% (H - Reference: <5.7%)
- BUN: 18 mg/dL (Reference: 7-20)
- Creatinine: 1.1 mg/dL (Reference: 0.7-1.3)
- eGFR: 78 mL/min (Reference: >60)
- Sodium: 140 mEq/L (Reference: 136-145)
- Potassium: 4.2 mEq/L (Reference: 3.5-5.0)
- Chloride: 102 mEq/L (Reference: 98-106)
- CO2: 24 mEq/L (Reference: 23-29)
- Calcium: 9.4 mg/dL (Reference: 8.5-10.5)

LIPID PANEL:

- Total Cholesterol: 218 mg/dL (H - Reference: <200)
- LDL Cholesterol: 132 mg/dL (H - Reference: <100)
- HDL Cholesterol: 48 mg/dL (L - Reference: >40)
- Triglycerides: 190 mg/dL (H - Reference: <150)

COMPLETE BLOOD COUNT:

- WBC: $7.2 \times 10^9/L$ (Reference: 4.5-11.0)
- RBC: $4.9 \times 10^{12}/L$ (Reference: 4.5-5.5)

- Hemoglobin: 14.8 g/dL (Reference: 13.5-17.5)
- Hematocrit: 44.2% (Reference: 38.8-50.0)
- Platelets: 245 x10⁹/L (Reference: 150-400)

LIVER FUNCTION:

- ALT (SGPT): 32 U/L (Reference: 7-56)
- AST (SGOT): 28 U/L (Reference: 10-40)
- Alkaline Phosphatase: 78 U/L (Reference: 44-147)
- Total Bilirubin: 0.8 mg/dL (Reference: 0.1-1.2)
- Albumin: 4.1 g/dL (Reference: 3.5-5.0)

THYROID:

- TSH: 2.4 mIU/L (Reference: 0.4-4.0)
- Free T4: 1.1 ng/dL (Reference: 0.8-1.8)

OTHER:

- Vitamin D, 25-OH: 42 ng/mL (Reference: 30-100)
- PSA: 1.8 ng/mL (Reference: <4.0)
- Uric Acid: 6.8 mg/dL (Reference: 3.5-7.2)

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MEDICAL HISTORY

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SURGICAL HISTORY:

- Appendectomy (2001) - uncomplicated
- Right knee arthroscopy (2015) - meniscus repair
- Colonoscopy (2024) - screening, benign polyp removed

FAMILY HISTORY:

- Father: Type 2 Diabetes, Hypertension, MI at age 62 (deceased at 74)
- Mother: Osteoarthritis, Hyperlipidemia (living, age 78)
- Brother: Hypertension (age 52)
- Sister: No significant medical history (age 48)
- Paternal Grandfather: Stroke (deceased)
- Maternal Grandmother: Breast cancer (deceased at 81)

SOCIAL HISTORY:

- Occupation: Software Engineer
- Tobacco: Former smoker, quit 2010 (15 pack-year history)
- Alcohol: Occasional, 2-3 drinks per week
- Exercise: Moderate, walks 30 minutes 3x/week
- Diet: Working on low-carb diet for diabetes
- Marital Status: Married, two children
- Stress Level: Moderate work-related stress

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IMMUNIZATIONS

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- Influenza Vaccine: October 2025 (annual)
- COVID-19 Vaccine: September 2025 (updated booster)
- Tdap: 2022 (due 2032)
- Pneumococcal (PCV15): 2024
- Shingles (Shingrix): 2024 (dose 1), 2024 (dose 2)
- Hepatitis B: Series completed 2020

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ASSESSMENT & PLAN

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1. TYPE 2 DIABETES - SUBOPTIMALLY CONTROLLED

- A1c 7.2%, goal is <7.0%
- Continue Metformin 1000mg twice daily
- Continue Januvia 100mg daily
- Reinforce diet and exercise
- Recheck A1c in 3 months
- Consider GLP-1 agonist if no improvement

2. HYPERTENSION - UNCONTROLLED

- BP 142/88 today, goal <130/80
- Increase Lisinopril from 20mg to 40mg daily
- Add Metoprolol Succinate 50mg daily
- Home BP monitoring twice daily
- Low sodium diet counseling provided

- Recheck BP in 2 weeks

3. HYPERLIPIDEMIA - IMPROVING BUT NOT AT GOAL

- LDL 132, goal <100 given diabetes
- Continue Atorvastatin 40mg nightly
- Continue Aspirin 81mg daily
- Diet counseling reinforced
- Recheck lipids in 3 months

4. OBESITY

- BMI 28.4, goal BMI <25
- Discussed weight loss strategies
- Referral to nutritionist placed
- Continue walking program
- Consider weight loss medication if no progress

5. CHRONIC LOW BACK PAIN

- Stable, mild symptoms
- Continue stretching exercises
- Acetaminophen PRN (avoid NSAIDs due to allergy)
- Physical therapy referral if worsens

FOLLOW-UP:

- Blood pressure recheck in 2 weeks
- Hemoglobin A1c recheck in 3 months
- Annual wellness exam in 12 months

- Call if BP symptoms (headache, vision changes, chest pain)

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SIGNATURES

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Electronically signed by:

Dr. Sarah Martinez, MD

Board Certified - Internal Medicine

Downtown Family Medicine

NPI: 1234567890

Date/Time: April 22, 2026 at 3:45 PM EST

This clinical summary has been reviewed and authenticated.

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END OF CLINICAL SUMMARY

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